



Clinical supervision in mental health

Policy code	<i>SMHS:49</i>
Original endorsement date	<i>1 December 2010</i>
Date of current endorsement	<i>11 July 2025</i>
Functional sub group	Mental health
Summary	South Metropolitan Health Service (SMHS) requires all clinical staff be engaged in a process of clinical supervision. The aim is to support the clinician in their professional environment with the goal of improving clinical practice and improving patient outcomes. All SMHS mental health services are advised to develop detailed protocols and procedures, as required, to inform and guide their local staff in the application of this policy.
Policy owner	Executive Director Mental Health
Contact	Carole Steiner
Review date	11 July 2028
NSQHS Standards	Standard 1: Clinical Governance
Status	<i>Active</i>

1. Background

1.1. About this document

SMHS requires all mental health clinical staff, Aboriginal mental health staff and Lived Experience Peer Workers to be engaged in a process of clinical supervision at a level appropriate to their qualifications and experience.

All SMHS mental health services will have site-based protocols and procedures, to inform and guide their staff in the application of this policy.

1.2. Key definitions

Term	Definition
Clinical Supervision	The process of two or more professionals (supervisor and supervisee) meeting formally to reflect on and review clinical situations and practice with the aim to support the clinician in their professional environment and enable teaching and learning at the point of care. The goal is to further develop knowledge and competence in the context of the clinician's experience and in providing safe, high quality, and appropriate care. Clinical supervision provides a formal process of support, reflection, learning and development.
Clinical Supervisor	An experienced clinician nominated to discuss casework and other professional issues in a structured manner. The clinical supervisor will have the qualifications and skills necessary to supervise the nominated area of clinical practice.
Line Manager	The person/manager directly responsible for an employee's performance within the organisation hierarchy.
Professional or Clinical Lead	Leads and coordinates the professional discipline and leads the promotion and maintenance of professional standards. Acts as an expert resource and provides consultation, education and supervision on complex mental health disorders at an advanced practice level to external practitioners and agencies.
Supervisee	Any member of each of the professions; medical, allied health and nursing, Lived Experience Peer Workers and Aboriginal mental health staff, employed to provide direct clinical care to mental health consumers.

2. Policy Statement

All SMHS mental health clinical staff will participate in, and keep a written record of, clinical supervision appropriate to:

- clinical role
- the requirements of their profession
- the requirements of the service

- statutory and other regulatory requirements

The purpose of clinical supervision is to:

- provide ongoing learning and clinical skill development
- provide support in coping with the demands of clinical work
- promote reflective practice and maintenance of professional and ethical standards
- ensure culturally competent care, including Aboriginal cultural responsiveness
- promote evidence-based practice.

Staff are required to refer to [Appendix A: SMHS Mental Health Clinical Supervision Standard of Practice and Procedure](#) or to specific discipline led procedures or protocols aligned to this policy.

Refer to the [WA Drug and Alcohol Office Clinical Supervision Handbook](#).

2.1. Out of scope

Components of professional development not addressed in this policy include, but are not limited to:

- Cultural consultation
- Cultural supervision
- Mentoring
- Coaching
- MDT Clinical Reviews
- Line Management
- Clinical Psychologist Registrars/Clinical Neuropsychologist Registrars
- Lived Experience Peer Worker Peer Supervision

Refer to [Appendix B Out of Scope](#) for more detail.

3. Roles and responsibilities

3.1. Clinical Supervisor

The clinical supervisor must meet the following minimum standards:

- Have at least two years clinical experience as a mental health professional and be approved by the relevant health board, as is required.
- Preferably be from the same profession as the supervisee.
- Preferably have attained a higher level of skills and experience than the supervisee.
- Have experience and / or training as a clinical supervisor.
- Be available and accessible

3.2. Supervisee

The clinical supervisee is encouraged to:

- Actively participate in clinical supervision, and preparation for each session by bringing cases or any issues they would want to discuss.

- Participate in informal evaluation of clinical supervision.
- Work with the supervisor to resolve any concerns or problems arising from a supervision relationship.
- Ensure that their line manager is aware and supports the clinical supervision sessions.

4. Clinical supervision for Lived Experience Peer Workers

Lived Experience Peer workers will participate in clinical supervision as per the wider clinical team. Clinical supervision for Lived Experience Peer workers should be provided by a clinician with a clear understanding of the guiding principles of peer practice, relevant clinical knowledge and skills and who is not the Lived Experience Peer workers Line Manager.

The Supervisor should have previous experience working alongside Lived Experience staff and/or have undertaken education or training to understand the guiding principles for Lived Experience Peer workforces. The Clinical Supervisor is an ally for the Lived Experience Peer worker and may undertake internal advocacy to support and champion the role.

Clinical Supervisors for Lived Experience Peer workers should be mindful of:

- reducing peer drift – when Lived Experience Peer workers shift towards a medical model of care and deviate from the practices that distinguish them as Lived Experience Peer workers.
- the potential for power imbalances and the need to create mutual, two-way learning relationships through the process of purposeful discussion.

Clinical supervision for Lived Experience Peer workers is different to Peer-to-Peer Supervision.

5. Minimum requirement for participation in clinical supervision

Each staff member who is engaged in providing direct clinical care, or Lived Experience Peer Support to patients of SMHS mental health is encouraged to participate in a minimum of one (1) hour formal clinical supervision per month (or pro rata based on FTE). This can occur either individually or within a group environment.

The Line Manager and/or Professional/Clinical Lead is to support staff to meet the recommended minimum of one (1) hour clinical supervision per month.

6. Documentation

Both supervisors and supervisees will keep a copy of the documentation related to clinical supervision including a log of the sessions completed. Refer to [Appendix C](#) or to specific discipline led documentation templates.

The supervisee is to maintain a log-sheet of their clinical supervision sessions, which records the date and duration of each session. A copy is to be kept by the supervisor and the log sheet is to be signed by both participants (supervisor and supervisee).

At the time of the supervisee's annual performance development review, the completed log-sheet is to be provided to the supervisee's line manager and professional lead as evidence of their participation in clinical supervision.

Both supervisors and supervisees will keep a copy of the documentation related to clinical supervision including a log of the sessions completed.

Clinical supervision arrangement documentation should also include:

- a statement about confidentiality and its limits in relation to clinical supervision and the process for managing inappropriate practice.
- storage and ownership of process notes negotiated between clinician and supervisor within the policy guidelines

7. Compliance monitoring

7.1. Compliance monitoring

Each SMHS site or Service Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and compliance risks and training requirements are managed.

All staff participating in a formal clinical supervision arrangement must:

- provide evidence of their participation to their line manager on request; and
- provide evidence of their participation to their line manager at the time of their annual Performance Management Review.
- provide evidence of their participation in any supervision internal or external to the service to their line manager on request.

Line managers or Professional/Clinical Leads are to keep a record of evidence of their staff participation in formal clinical supervision arrangements.

8. Legislation

- [Mental Health Act 2014](#)
- [Alcohol and Other Drugs Act 1974](#)

8.1. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

9. Related Documents

9.1. Department of Health policies

- [MP 0095/18 Clinical Handover Policy](#)

9.2. SMHS Policies

- [SMHS: 62 Professional Development Review](#)

9.3. Other documents

- [National Practice Standards for the Mental Health Workforce \(2013\)](#)
- [National Safety Quality Health Service Standards \(20121\)](#)
- [Chief Psychiatrists Standards for Clinical Care](#)
- [Mental Health Commission Lived Experience \(Peer\) Workforces Framework](#)

9.4. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP: V3.0	16/07/2025	11/07/2025	11/07/2028	July 2028	Triennial Review

9.5. Approval

Approval by	Initial approval position title
Approval date	X Month Year

10. Appendix A: SMHS Mental Health Clinical Supervision Standard of Practice and Procedure

10.1. Standard

Clinical supervision sessions will consist of discussion of the supervisee's clinical work and interactions with mental health consumers and will involve consideration of issues such as:

- the nature of the therapy/care being provided
- the clinical skill utilised to provide the therapy/care
- transference and countertransference issues
- therapeutic boundaries issues
- ethical and professional issues
- cultural, health and wellbeing needs of Aboriginal people from ethnoculturally and linguistically diverse backgrounds and people with sexual, gender and bodily diversity.

Both supervisors and supervisees will keep a documented record of the process [Appendix C](#).

10.2. Format

Clinical supervision specific to an area of practice, with instructive critique on discipline specific care, can be provided in many different forms, depending on the clinician's preference and organisational context.

- **Individual one to one**
- **Group:** A situation of two or more clinicians in an agreed clinical supervision process.
- **Peer:** A situation where peers of the same profession confer with one another on key topics of their professional everyday work in order to provide effective or alternate solutions in dealing with difficult situations. There is no distinguishable supervisor role; the level of engagement is equally distributed.
- **Cross Discipline:** One-to-one or group clinical supervision with more than one professional discipline involved.
- **Reflective Practice:** A process of thinking clearly, honestly, deeply, and critically about any aspect of our professional practice. Reflective practice is an effective process to develop self-awareness and facilitate changes in professional behaviour. When reflective practice occurs as a component of supervision it can be in relation to reflecting on the supervisees clinical work. While clinical supervision is expected to include reflective practice, reflective practice alone does not constitute clinical supervision and should be done in conjunction with one of the above-mentioned clinical supervision approaches.

10.3. Establishing a Clinical Supervision Agreement

Where the clinical supervisor is also the line manager / line supervisor, this relationship is usually outlined in the JDFs of both parties. In such cases, this distinction should be clearly outlined in clinical supervision arrangement documentation, to differentiate between instances where the professional relationship is between line manager and staff member, and where the professional relationship is between supervisor and supervisee.

It is the responsibility of the Line Manager or delegate to ensure that suitable arrangements for clinical supervision in place for all staff who report up, directly or indirectly, to their position.

10.4.Choosing a supervisor

Where the clinical supervision relationship is not defined in the JDF, a staff member may prefer to seek a clinical supervision arrangement with someone another appropriate discipline than their line manager/line supervisor.

The staff member seeking to set-up a clinical supervision agreement may approach another mental health professional to provide their clinical supervision. The mental health professional must meet minimum standards of competence to be suitable as a supervisor.

A clinical supervisor will:

- have at least two years clinical experience as a mental health professional and be approved by the relevant health board, as is required.
- have the qualifications and skills necessary to supervise the nominated area of clinical practice.
- preferably have attained a higher level of skills and experience than the supervisee.
- have experience and / or training as a clinical supervisor.
- be available and accessible.

10.5.Approval for supervision

When a supervisor has been found, and they have agreed to provide clinical supervision, the supervisee will approach their line manager or Professional/Clinical Lead for their approval of the arrangement.

10.6.Documenting an agreement

When a supervisor has been found and the approval of the line manager / line supervisor has been obtained, proper documentation of the arrangement must be agreed to both/all parties with each person retaining a copy for their records. Refer to [Appendix C](#).

10.7.Keeping a record of sessions

A record must be kept of the schedule of clinical supervision meetings with both/all participants retaining a copy for their records.

The schedule will be kept as a log and will be dated and signed by both/all participants after each session. Refer to [Appendix C](#).

Supervision session records will be kept confidential and not disclosed for any improper purpose unless disclosure is required by law or HSP Policy to be provided.

10.8. Conflict within the Clinical Supervision Arrangement

If any conflict arises within the clinical supervision arrangement, both parties should attempt to negotiate a mutually acceptable resolution to any issues.

If issues cannot be resolved satisfactorily this way, then either or both parties should refer the matter to the Line Manager or Professional/Clinical Lead to assist in resolution of the matter.

If an agreement cannot be reached between the parties the endorsed [SMHS Grievance Resolution Procedure for Employees and Managers](#) should be used.

10.9. Changing a supervisor

For a variety of reasons, it may sometimes be necessary or appropriate to change the clinical supervisor. Proposed changes to the clinical supervisor / supervision arrangement should be discussed with the Line Manager or Professional/Clinical Lead in advance and the Line Manager or Professional/Clinical Lead should approve any such changes.

10.10. Performance or risk issues

Should major performance or risk issues become apparent during the clinical supervision process, the clinical supervisor should advise the line manager or Professional/Clinical Lead (if they are not the same person).

Supervisees should be made aware at the beginning of the supervision arrangement that this may occur as the organisation has a responsibility to ameliorate any risk issues, and the line manager or Professional/Clinical Lead has the responsibility to constructively manage/resolve any performance or risk issues to achieve a positive outcome.

10. Appendix B: Out of Scope

Cultural consultation	Encourages non-Aboriginal clinicians and staff to maintain proficiency and reflect on their work practice and relationship with Aboriginal people, their carers, families, and communities. It is recommended particularly for non-Aboriginal clinicians who regularly work with Aboriginal people, their carers, families, and communities. Refer to the SMHS Patient Centred Cultural Care Guidelines
Cultural supervision	A group session facilitated by a senior Aboriginal clinician that provides Aboriginal mental health workers an opportunity to discuss cultural and clinical concerns in a culturally safe space. Refer to the Chief Psychiatrists Standards for Clinical Care .
Mentoring	Creating relationships where productive conversations and learning can occur between two colleagues, while providing an opportunity to reflect on career challenges, discuss strategies and celebrate achievements
Coaching	A systematic and individualised approach that aims to enhance performance, facilitate learning, and unlock potential. It is a supportive process that empowers individuals to overcome challenges, build new skills, and achieve their goals.
MDT Clinical reviews	A formal process to monitor clinical assessment and treatment progress and provide an opportunity to identify any barriers to discharge early in the episode of care and plan interventions to address these barriers.
Line management	The role that the line manager has in operational decision-making and in overseeing the work of the staff member to ensure they are operating within their scope of practice.
Clinical Psychologist Registrars / Clinical Neuropsychologist Registrars	AHPRA/Psychology Board of Australia requires weekly supervision with a Board Approved Supervisor to demonstrate that competencies relevant to the area of practice have been met at a level consistent with the depth and expertise expected of an entry-level endorsed psychologist (endorsed pathway). A principal or secondary supervisor for a registrar program must be a registered psychologist and Board approved supervisor. Requirements for Board approved supervisors are detailed in the Board's Guidelines for supervisors . Under these guidelines, principal supervisors of registrars in an area of practice must have held endorsement in that area of practice for at least two years Supervision for registrars must be provided by the principal supervisor (within area of practice endorsement) for at least 50% and up to 100% of the total supervision in the registrar program with documentation submitted to the Board.
Lived Experience Peer Worker Peer Supervision.	Supervision that is undertaken peer to peer. It is described as a safe space to explore strengths, problem-solve challenges and tensions and experience empathy and validation. Peer supervision keeps Lived Experience Peer workers connected to the under-pinning values and principles of the discipline and avoiding drifting into clinical (or other non-lived experience) perspectives and ways of working. Peer

	supervision may occur internally, externally, or both. Refer to the Mental Health Commission Lived Experience (Peer) Workforces Framework .
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10. Appendix C: Clinical Supervision Agreement / Record of Sessions

Agreement

Supervisee:		Contact details:	
Supervisor:		Contact details:	
Commencement date:		COPY TO BE FORWARDED TO LINE MANAGER ON REQUEST / PDR	
<i>We agree to keep all discussion in clinical supervision confidential. There is a legal duty of care that may override confidentiality in exceptional circumstances. Such circumstances would be if the supervisee is describing unsafe, unethical or illegal practice and unwilling to go through appropriate procedures to address these after initial discussion between supervisor/supervisee.</i>			
Supervisee signature:		Supervisor signature:	

Record of sessions

Planned date	Session held?	Key area discussed	Agreed actions/comments	Type of contact	Date completed

This document can be made available in alternative formats on request.

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